



A predictable rise

Inflation will remain high in the foreseeable future

A rise in India's July retail inflation was a foregone conclusion, but its modest increase, at 4.45%, up from 4.38% in June, is still the highest in 19 months, since December 2024. This is the second consecutive month that retail inflation has stayed above the Reserve Bank of India (RBI)'s 4% target, even while remaining comfortably within its 2%-6% tolerance range. Predictably, it has again been driven by food, fuel and transport, even as core inflation, excluding precious metals, has remained below 3%. What is telling is the extent of rural, food-led inflation, which rose from 5.45% in June to 5.79% in July, while urban food inflation decreased marginally from 5.09% to 5.05%. Staples such as onion (22.54%), garlic (35.36%) and ginger (83.62%) fuelled the rise, even as potato (-16.56%) and tomato (-4.59%) moderated. Transport, however, continues to have a wider impact, pointing to elevated input-cost pressures. Transport inflation quickened to 4.43% in July from 4.31% in June, while the crucial subdivision, transport services for goods, rose from 7.70% to 7.77%. Despite the July 1 cut in commercial LPG prices of about ₹183, food and beverage serving services inflation quickened to 7.75% in July, indicating that restaurants are yet to recoup revenues and margins lost following the steep operating costs from March through May. Commercial LPG was cut by a further ₹202 on August 1, but this is unlikely to immediately bring down menu prices.

The monsoon remains a concern, with parts of western, central and southern India remaining rain-deficient. Precious metals inflation, particularly gold (32.98%) and silver (109.84%), moderated, but remains extraordinarily high. Crude prices were relatively stable during the July CPI reference period, but began rising again in August. More worryingly, Ukraine-related disruptions around Russia's Black Sea export infrastructure, particularly Novorossiysk, could raise freight and risk premiums for Russian crude. Russia supplied nearly half of India's crude imports in June, making such disruptions relevant to India's landed energy costs. The rupee also depreciated by about 1.6% between the June 15 and July 15 CPI reference dates, further amplifying imported inflation. In the background, there are signs of weakening economic momentum, with the HSBC composite PMI showing a sharp fall from 57.1 in June to 54.3 in July, its weakest expansion since March 2022. While the PMI is a high-frequency indicator and need not reflect a long-term trend, it is nevertheless worth taking note of. The RBI's Monetary Policy Committee, which held the repo rate at 5.25% for the fourth consecutive meeting in August, is therefore likely to remain on hold through the second quarter of FY27, as it weighs persistent supply-side inflation against weakening economic momentum.

Sticks and goals

India might see its best finish in 50 years in hockey World Cup

When India celebrates its 80th Independence Day, its men's hockey team will open its World Cup campaign against Wales in Amstelveen, aiming to end a 51-year medal drought. Because of hockey's emotional connection with Indians, the wait has been agonising for the eight-time Olympic champion. It won its last medal when the Ajit Pal Singh-led side lifted the trophy in Kuala Lumpur in 1975 during the pre-synthetic turf era. Trust in the present Harmanpreet Singh-captained side has grown after India broke a 41-year Olympic jinx with bronze in Tokyo in 2021 and retained it in Paris 2024. Under coach Craig Fulton, the team won Asian Games gold in 2023, Olympic bronze in 2024, and the Asia Cup in 2025, with the last one helping it qualify for the World Cup. Despite finishing eighth in the FIH Pro League 2025-26, India showed positive signs by beating World champion Germany and Olympic champion the Netherlands. After experiencing experimentation, defeats and two controversies – involving most-capped player Manpreet Singh, and the unwarranted decision to change the jersey from the familiar blue to saffron, the squad has regrouped for its first World Cup following the retirement of legendary custodian P.R. Sreejesh. After Wales, the eighth-ranked side faces a formidable England and traditional rival Pakistan in the pool.

Comprising two-time Olympic medallists and promising youngsters, the team, placed sixth and ninth in the last two editions, now targets a higher finish in the World Cup before embarking next month on a mission to retain the Asian Games gold, which will earn it a berth in the 2028 Olympics. The Salima Tete-led women's team has rebuilt under Dutchman Sjoerd Marijne after turmoil caused first by the exit of his predecessor and then by an alleged sexual harassment case involving a former Hockey India official. Marijne, who took India to its best-ever fourth in Tokyo in his first stint, guided it to World Cup qualification and Nations Cup victory. Despite the Hockey India League experience and the exposure to foreign players, the team of five 200-cap players and 11 debutants may find it tough to match 1974's fourth-place finish. Ranked ninth, India plays Olympic silver medallist China on August 16, then South Africa and higher-ranked England. It will target a better position than 2022's ninth before switching focus to the Asian Games. For the first time, the World Cup for both genders involving 32 teams altogether, with a new and tricky format, is co-hosted by Belgium and the Netherlands. Both the Indian teams must play as cohesive units to build towards Los Angeles 2028.

India marked Doctors' Day on July 1, but the occasion demands that we continue to look beyond well-deserved expressions of gratitude and reflect on the structural foundations that sustain public confidence in health care. Trust in medicine is frequently thought of as a sacred, isolated bond between patient and doctor. In reality, that trust is forged much earlier – within the institutions that regulate care, the systems that deliver it, and the transparency that connects them.

The limitations of this system are readily observable. This writer tried the simple exercise of verifying a doctor's credentials on the Tamil Nadu Medical Council (TNMC) website. What should have been a straightforward search turned out to be unexpectedly challenging. The system required specific details, such as the doctor's exact registration number, which are not easily available to the public. Whether something obvious was missed or the website was simply not designed with the public – or even a colleague concerned – in mind, the experience pointed to a larger question: how much does institutional transparency shape public faith in medicine?

How institutions build trust

In many western countries, public trust is maintained through highly accessible regulatory frameworks. For example, the United Kingdom's General Medical Council hosts a publicly searchable register where anyone can quickly verify a doctor's qualifications, registration status, and any history of regulatory restrictions. Similarly, the provincial Colleges of Physicians and Surgeons in Canada require minimal identifying information to view a physician's professional profile. These organisations explicitly signal that professional standards are being ensured through peer review, continuing education, and clear processes for handling complaints. This transparency does more than just share data. It reassures people that medical practice is open to independent scrutiny, creating a safe environment where individual doctor-patient relationships can thrive.

India's clinical landscape is evolving admirably, with a workforce that is known for its excellent clinical acumen despite working under considerable constraints. While the National Medical Commission has taken steps to improve medical education and professional standards in recent years, public-facing regulatory transparency remains uneven. State Medical Councils, including the TNMC, hold digital registers, but the ease with which a regular patient can navigate them varies. As India continues its digital transformation, making these regulatory portals genuinely user-friendly is a



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Transparency across regulatory bodies, hospital systems, and clinical care can strengthen public faith and trust

Europe's AI rules may become India's opportunity

The Government of India has recently indicated that it is considering a standalone legislation to govern Artificial Intelligence (AI). However, the pressure on the AI supply chain is already felt due to the European Union (EU)'s AI Act which was entered into force in August 2024 and became applicable on August 2, 2026. The Act has adopted a risk-based approach to artificial intelligence ("AI") regulation – prohibiting certain AI systems, regulating high-risk ones, and imposing lighter checks for limited-risk use cases.

While most Indian firms are likely aware that the Act applies when their AI systems produce results in Europe, there is a more consequential story that goes beyond a compliance checklist.

A post-approval issue

The Act was drafted with a particular picture in mind, and that picture does not match how India's technology industry actually works. This mismatch, more than the law's long reach, is what Indian companies should be watching. The Act assumes that software, once built and approved, is sold as a finished product. India's technology industry has never worked that way. The devil lies in the legal consequences when an AI is updated post-approval.

Before a 'high-risk' AI system, used for sensitive decisions such as hiring or education, can enter the European market, it must clear a 'conformity assessment' under Article 43: proof that it meets the law's standards on testing, documentation and human oversight. For most such systems the provider assesses itself against the Act's criteria and signs its own declaration; only a narrow set, mainly certain biometric tools, must be checked by an independent body. Either way, once the box is ticked, the system can run undisturbed. However, if the system is "substantially modified", the process must be repeated. A substantial modification is a change



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India could build an industry around the paperwork and proof required under the European Union's Artificial Intelligence (AI) Act

not contemplated at the time of the original assessment, one that affects its compliance or alters its intended purpose.

In June 2026, the EU gave final approval to a package easing its own timelines, pushing the compliance deadline for these standalone high-risk AI systems to December 2027 and to August 2, 2028 for high-risk AI-embedded in regulated products. It retained a grandfathering clause under which systems already on the market before this date are exempted from the Act's obligations until substantially modified. A foreseen change examined during the original conformity assessment does not trigger a fresh one, while an unanticipated change likely does.

This distinction is not just a technicality. It would likely favour business models built on predictable product road maps. Providers of standardised AI products can assess planned upgrades during the initial assessment. Businesses that rely on bespoke services, offering competitive adaptability to individual client needs, may find it harder to demonstrate that future changes fall within the scope of the original assessment.

Unplanned improvement and regulation

India's technology industry, from the large IT services firms to the global capability centres in Bengaluru and Hyderabad, runs on exactly this kind of responsive adaptation. The Act also tethers liability to firms that substantially modify someone else's high-risk system. Such firms may be treated as the provider of the modified system, inheriting every one of the original maker's obligations. Thus, for an industry whose very promise is improvement on demand, an unplanned improvement that changes an AI system's intended purpose may trigger a fresh regulatory exercise.

Yet, the Act also presents an opportunity. High-risk compliance is operationalised through

profession – a space where high-stakes decisions are made in moments of profound human vulnerability. Here, transparency transitions from policy into a purely human interaction.

Modern patients increasingly expect clarity regarding who is treating them and why specific clinical decisions are chosen. This means explaining a diagnosis in plain language, detailing the rationale behind investigations, discussing therapeutic alternatives, and being transparent about costs and complications. Providing proactive, periodic updates on a patient's progress – or lack thereof – is often the single most effective way to ease a family's distress.

This shift toward shared decision-making recognises that transparency in treatment is about relationships, not just data. However, if transparency is to be a core part of modern medicine, it must be integrated into medical training. This cannot be taught in a lecture hall alone; it must be actively modelled by senior doctors during everyday clinical practice and teaching. By embedding this culture of communication early, we prepare the next generation of physicians to navigate increasing public expectations.

In my experience, the majority of conflicts in health care do not arise from the medical decisions themselves, but from how those decisions were communicated and understood. When patients feel genuinely heard and informed, the bond is strengthened. When communication is poor, even appropriate care may be perceived as inadequate.

A final reflection

Doctors need safe, respectful, and dignified environments in which to work. Patients require reassurance that the care they receive is both ethical and competent. These are not competing demands; they are mutually reinforcing truths.

Trust in medicine is built collectively by what our institutions make visible, how our clinical care systems function, and how doctors and patients talk to one another. When transparency runs through all three layers, it stops being an abstract ethical principle and becomes part of everyday practice.

Doctors' Day has passed, words of appreciation are important. But the days after also invite a structural look at our systems. At a time when the medical profession is practising under unprecedented strain, enhancing transparency in regulatory frameworks, clinical corridors, and individual consultation rooms is the most meaningful way to protect the integrity of the profession. Trust is not an entitlement to be claimed; it is a professional pact that must be continuously renewed through transparency.

paperwork and proof. Most providers will be permitted to carry out that conformity assessment themselves, but they will do so against harmonised technical standards still being drafted. The underlying work – governance measures, technical documentation, testing regimes – has to be done in volume. The scale of this obligation is likely to generate demand for quality legal and technical professionals, which India can provide. Indian professional services firms have supported clients across data protection, financial regulation and technical assurance. They can provide regulatory capability for the Act too.

Tap the trade agreement

A longer-term vision extends beyond compliance services. The Act creates a pathway for conformity assessment bodies established in third countries to be recognised and perform the functions of notified bodies, where the EU has concluded an appropriate agreement, and those bodies satisfy the requirements. This recognition is a question for treaty negotiation, and India has just signed a treaty capable of carrying it.

The India-EU Free Trade Agreement that was concluded in January includes standing machinery and regulatory cooperation provisions. If India can secure institutional arrangements as part of its partnership with the EU, it could become not just a supplier of AI compliance services but also a participant in the EU's conformity assessment ecosystem. Going ahead, it would provide the legal gateway through which qualified Indian conformity assessment bodies could eventually perform functions recognised under the Act. Europe has written itself a mountain of compliances and India has the capacity to do it. There is a version of this where the rules India fears become the work it sells. Which version arrives is still being decided, and not for much longer.

LETTERS TO THE EDITOR

What the JPC must ensure

The concerns surrounding the proposed Foreign Contribution (Regulation) Act amendments extend beyond their impact on civil society organisations. The deeper issue is whether regulatory power can be exercised without adequate safeguards for due process. Preventing the misuse of foreign contributions is a legitimate state objective, but automatic forfeiture of assets, denial of a hearing before non-renewal, and

the absence of an effective appeal against such decisions risk making executive discretion disproportionate to the alleged violations. The Joint Parliamentary Committee should ensure that national security and financial accountability are balanced with natural justice, transparency, and the protection of legitimate institutional property (Front page, August 13).

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The next horizon

The reported 'leadership transition' at the Tata Group should not be viewed merely as the departure of an individual. As markets, technologies and challenges change, institutions too need fresh thinking and new energy. The transition could also create an opportunity for a leader with a different perspective, greater risk appetite and the ability to shape the next chapter for the Tatas. The challenges ahead – from

artificial intelligence and semiconductors to green energy, electric mobility and global supply chains – are markedly different from those of the past. The real question, therefore, is whether the 'new Chandra' can illuminate the Tatas' next horizon (Business page, August 13).

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English and India

While the upper and middle classes have long

recognised the importance of English, the aspirations of the marginalised, the rural poor and those who have historically lacked access to opportunities are equally significant. Across India, parents from these sections are desperate to send their children to English-medium schools. For them, English is not a symbol of colonial domination; it is a royal road to social mobility, opportunity and emancipation from inherited constraints. Earlier

generations of Indians used English to transcend social and economic barriers and gain access to education, employment and public life (Editorial page, August 11). Does the English language belong to India? The aspirations of successive generations of Indians have already answered that question.

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